Eribulin · Days 1 and 15 of 28

National Cancer Control Programme (NCCP) 00743 · Source reconciled; independent clinical, pharmacy and patient/PPI review pending

PRACTICAL TREATMENT GUIDE

Your treatment and access plan

Eribulin on Days 1 and 15 of each 28-day cycle. Days 8 and 22 are planned treatment-free weeks. This alternative schedule requires a specific discussion and consent.

STEP 1

Before treatment

full blood count (FBC), kidney and liver tests at baseline and the start of each cycle. Confirm count recovery for each administration. Correct low potassium, calcium and magnesium before treatment and monitor periodically.

STEP 2

Plan venous access

A cannula, peripherally inserted central catheter (PICC) or implanted port may be used. A port is not automatically required; insertion and removal are separate procedures with individual instructions.

STEP 3

On the treatment day

Confirm the medicines due that day, any premedicines and the reviewed blood results. Tell staff immediately about pain, burning, rash or breathlessness.

STEP 4

After the visit

No routine take-home infusion pump is required for this regimen. Keep line-care, urgent-contact and next-review instructions.

Visit length

Eribulin is given over 5 minutes in this source. Preparation and blood review add to the visit; doses are on Days 1 and 15.

Line care

A PICC or port has its own care plan. Report fever, redness, swelling, pain, leakage or line damage; do not remove or adjust it yourself.

Treatment changes

A delayed, omitted or changed dose needs an updated plan from the treating team. Do not move doses or restart on your own.

Support for this visit

No routine hypersensitivity premedication is specified. Low-risk antiemetic support is reviewed separately, including QT interactions.

Sources: NCCP 00743 v3 · official regimen · eviQ · implanted port patient information · eviQ · PICC patient information. Source reconciled; independent clinical, pharmacy and patient/PPI review pending.